



Name : Mrs. KEERTHANA N
Age / Gender : 33 Year(s)/ Female
Contact No. : 7604833651
Address : FLAT F, 2ND FLOOR, PLOT NO 357, 358, 359, ...
Pin code : 600100

VID No. : 250217516247955
PID No. : P51025566377801
Referred by : DR.KOKILAVANI
Registered On : 29/03/2026 9:07 AM
Collected On : 29/03/2026 7:41AM
Reported On : 29/03/2026 5:27 PM

CBC Haemogram

Investigation	Observed Value	Unit	Biological Reference Interval
<u>Erythrocytes</u>			
Haemoglobin (Hb) (Photometric measurement)	10.4	gm/dL	12.5-16
Erythrocyte (RBC) Count (RBC Pulse Height Detection method)	3.54	mill/cu.mm	4.2-5.4
PCV (Packed Cell Volume) (Calculated)	31.5	%	37-47
MCV (Mean Corpuscular Volume) (Automated - Pulse Height Detection)	89.1	fL	78-100
MCH (Mean Corpuscular Hb) (Calculated)	29.3	pg	27-31
MCHC (Mean Corpuscular Hb Concn.) (Calculated)	32.9	g/dL	32-36
RDW (Red Cell Distribution Width)	15.5	%	11.5-14.0
<u>Leucocytes</u>			
Total Leucocytes (WBC) Count	9400	cells/cu.mm	4000-10500
Absolute Neutrophils Count	6947	cells/cu.mm	2000-7000
Absolute Lymphocyte Count	1692	cells/cu.mm	1000-3000
Absolute Monocyte Count	639	cells/cu.mm	200-1000
Absolute Eosinophil Count	94	cells/cu.mm	20-500
Absolute Basophil Count	28	cells/cu.mm	20-100
Neutrophils	73.9	%	40-80
Lymphocytes	18.0	%	20-40
Monocytes	6.8	%	2.0-10
Eosinophils	1.0	%	1-6
Basophils	0.3	%	0-2
<u>Platelets</u>			
Platelet count	237	10 ³ /μL	150-450
MPV (Mean Platelet Volume)	8.7	fL	6-9.5
PCT (Platelet Haematocrit)	0.207	%	0.2-0.5
PDW (Platelet Distribution Width)	16.3	%	9-17

EDTA Whole Blood

Danita Edwin
DR.DANITA EDWIN
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Consultant Pathologist
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Investigation

Observed Value

Unit

Biological Reference Interval

- Tests done on Automated Five Part Cell Counter. (WBC,Platelet count by impedance method/DC detection,RBC by pulse height detection method, HB by Automated - Photometric Measurement, WBC differential by VCS technology other parameters calculated)

All Abnormal Haemograms are reviewed confirmed microscopically. Differential count is based on approximately 10,000 cells.

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Sample Collected At:

RA-MRS REKHA A (PALLIKARRI 2), FALT NO C 2 I TRANQUIL ACRES
PHASE 2 200 FEET MMRD ROAD, KANCHIPURAM CHENNAI...

Processing Location :

Processing LocationMetropolis Health care Ltd.#236,PR
Plaza,Velachery MainRoad,Selaiyur,chennai-600073



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Investigation

Glucose Plasma (75gms)

(Fluoride Plasma,Hexokinase)

Observed Value

106

Unit

mg/dL

Biological Reference Interval

Normal: 80-139

Impaired glucose tolerance: 140-199

Diabetes mellitus: ≥ 200

(on more than one occassion)

(American diabetes association
guidelines 2018)

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Investigation	Observed Value	Unit	Biological Reference Interval
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Haemogram Advanced Profile - 1

ESR (Erythrocyte Sedimentation Rate)	75	mm/hr	0-20
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(EDTA Whole Blood,Westergren method)

Method: Automated Westergren

Interpretation:

- It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
- It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
- It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

Remark : ESR Performed using capillary photometric aggregation (for automated analysis) & westergrens (for manual testing).

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Investigation	Observed Value	Unit	Biological Reference Interval
<u>Thyroid Profile - 2</u>			
(Serum, Electrochemiluminescence immunoassay (ECLIA))			
FT3 (Free Triiodothyronine)	2.97	pg/mL	2.0-4.4 First Trimester :2.46 - 3.49 Second Trimester : 2.09 - 3.55 Third trimester : 2.01 - 3.27
FT4 (Free Thyroxine)	1.03	ng/dL	0.93-1.7 First Trimester : 0.7-2.0 Second Trimester : 0.5-1.6 Third Trimester : 0.5-1.6
TSH (Thyroid Stimulating Hormone) - Ultrasensitive, Serum	4.97	µIU/mL	0.54-5.3 First Trimester : 0.33-4.59 Second Trimester : 0.35-4.10 Third trimester : 0.21-3.15

INTERPRETATION

TSH	T3 / FT3	T4 / FT4	Suggested Interpretation for the Thyroid Function Tests Pattern
Within Range	Decreased	Within Range	Isolated Low T3 - often seen in elderly & associated Non-Thyroidal illness. In elderly, the drop in T3 level can be up to 25%.
Raised	Within Range	Within Range	- Isolated High TSH, especially in the range of 4.7 to 15 mIU/ml, is commonly associated with Physiological & Biological TSH Variability. - Subclinical Autoimmune Hypothyroidism - Intermittent T4 therapy for hypothyroidism - Recovery phase after Non-Thyroidal illness
Raised	Decreased	Decreased	- Chronic Autoimmune Thyroiditis - Post thyroidectomy, Post radioiodine - Hypothyroid phase of transient thyroiditis
Raised or within Range	Raised	Raised or within Range	- Interfering antibodies to thyroid hormones (anti-TPO antibodies) - Intermittent T4 therapy or T4 overdose - Drug interference - Amiodarone, Heparin, Beta blockers, steroids, anti-epileptics
Decreased	Raised or within Range	Raised or within Range	- Isolated Low TSH - especially in the range of 0.1 to 0.4 often seen in elderly & associated with Non-Thyroidal illness - Subclinical Hyperthyroidism - Thyroxine ingestion
Decreased	Decreased	Decreased	- Central Hypothyroidism - Non-Thyroidal illness - Recent treatment for Hyperthyroidism (TSH remains suppressed)
Decreased	Raised	Raised	- Primary Hyperthyroidism (Graves' disease), Multinodular goitre, Toxic nodule - Transient thyroiditis: Postpartum, Silent (lymphocytic), Postviral (granulomatous, subacute, DeQuervain's), Gestational thyrotoxicosis with hyperemesis gravidarum
Decreased or within Range	Raised	Within Range	- T3 toxicosis - Non-Thyroidal illness

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References: 1. Interpretation of thyroid function tests. Dayan et al. THE LANCET • Vol 357 • February 24, 2001
2. Laboratory Evaluation of Thyroid Function, Indian Thyroid Guidelines, JAPI, January 2011, vol. 59

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Routine Examination Profile - Urine

Investigation	Observed Value	Unit	Biological Reference Interval
<u>Routine Examination Profile - Urine</u>			
<u>General Examination</u>			
Colour (Naked Eye Examination)	Pale Yellow		Pale Yellow
Transparency (Appearance) (Naked Eye Examination)	Clear		Clear
Reaction (pH) (Automated Photoelectric colorimetry)	6.0		4.5-8
Specific gravity (Automated Photoelectric colorimetry)	1.015		1.010-1.030
<u>Chemical Examination</u>			
Urine Protein (Albumin) (Sulpho salicylic acid method)	Absent		Absent
Urine Ketones (Acetone) (Rothera's method)	Absent		Absent
Urine Glucose (sugar) (Benedicts Test)	Absent		Absent
Bile pigments (Fouchet's method)	Absent		Absent
Bile salts (Hay's method)	Absent		Absent
Urobilinogen (Ehrlich method)	Normal		Normal
Nitrite (Automated Photoelectric colorimetry)	Negative		Negative
<u>Microscopic Examination</u>			
Red blood cells	Absent	/hpf	Absent
Pus cells (WBCs)	2 - 3	/hpf	0-5
Epithelial cells	1 - 3	/hpf	0-4
Crystals	Absent		Absent
Cast	Absent		Absent
Amorphous deposits	Absent		Absent

AK

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Investigation	Observed Value	Unit	Biological Reference Interval
Bacteria	Absent		Absent
Trichomonas Vaginalis	Absent		Absent
Yeast cells	Absent		Absent

Note : 1.

Chemical examination through Dipstick includes test methods as Protein (Protein Error Principle), Glucose (Glucose oxidase-Peroxidase), Ketone (Legals Test), Bilirubin (Azo- Diazo reaction), Urobilinogen (Diazonium ion Reaction) Nitrite (Griess Method). All abnormal results of chemical examination are confirmed by manual methods.

2.

Pre-test conditions to be observed while submitting the sample- First void, mid-stream urine, collected in a clean, dry, sterile container is recommended for routine urine analysis, avoid contamination with any discharge from vaginal, urethra, perineum, as applicable, avoid prolonged transit time & undue exposure to sunlight.

3.

During interpretation, points to be considered are Negative nitrite test does not exclude the urinary tract infections, Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs

.4.

All urine samples are checked for adequacy and suitability before examination.

-- End of Report --

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